

CITATION: Antinozzi v. Andrews, 2011 ONSC 3296

COURT FILE NO.: CV-08-092104

DATE: 20110719

CORRIGENDA: 20110803

SUPERIOR COURT OF JUSTICE - ONTARIO

RE: Kerry-Anne Antinozzi, Plaintiff

AND: Bradley Andrews, Defendant

BEFORE: The Honourable Mr. Justice C. Boswell

COUNSEL: M. Elkin, for the Plaintiff

H. Goddard and K. Lasko, for the Defendant

HEARD: May 30, 2011

REVISED ENDORSEMENT

**The text of the original endorsement has been corrected with the text of corrigendum
(released today's date)**

OVERVIEW:

[1] In this action the Plaintiff sought damages for injuries allegedly suffered in a motor vehicle accident on July 20, 2007. The trial of the Plaintiff's claims proceeded before a jury at Newmarket. The Defendant moved, at the outset of the trial, for a declaration that the Plaintiff's claim for non-pecuniary losses is barred by section 267.5(5) of the *Insurance Act*.¹ The Defendant's motion was argued at the conclusion of the evidence and the commencement of deliberations by the jury. Prior to the return of the jury verdict, I advised counsel orally that in my view the Plaintiff's action was not statute barred and I dismissed the Defendant's threshold motion for reasons to be provided in writing. These are my reasons.

STATUTORY FRAMEWORK:

[2] Pursuant to section 267.5 of the *Insurance Act*, a person injured in a motor vehicle accident in Ontario is prohibited from suing for non-pecuniary damages unless he or she meets a threshold test established by subsection 5 of section 267.5, which provides as follows:

(5) Despite any other Act and subject to subsection (6), the owner of an automobile, the occupants of an automobile and any person present at the

¹ R.S.O. 1990, c.I.8

incident are not liable in an action in Ontario for damages for non-pecuniary loss, including damages for non-pecuniary loss under clause 61 (2) (e) of the *Family Law Act*, from bodily injury or death arising directly or indirectly from the use or operation of the automobile, unless as a result of the use or operation of the automobile the injured person has died or has sustained,

- (a) permanent serious disfigurement; or
- (b) permanent serious impairment of an important physical, mental or psychological function.

[3] The general import of the threshold requirement is to limit the right of individuals involved in motor vehicle accidents to maintain tort actions against at fault parties. The threshold is just one piece of an overall scheme designed to control automobile insurance premiums in Ontario.²

[4] Threshold requirements have been in place in Ontario for roughly twenty years and have evolved over time. The most recent refinements to the threshold test are contained in Regulation 381/03, commonly referred to as “Bill 198”. Bill 198 came into force on October 1, 2003 and applies to motor vehicle accidents occurring after that date. The relevant provisions of Bill 198 are attached as Appendix “A”. They provide definitions for the terms “permanent”, “serious impairment”, and “important function”. They also provide direction as to what types of evidence must be called to establish an exemption under sub-paragraph 276.5(5)(b).

[5] The generally accepted view appears to be that the purpose of Bill 198 was to tighten up the threshold, thereby limiting the number of injured persons able to sue³, by raising the bar for prospective plaintiffs.⁴

[6] In this instance, there is no allegation that the Plaintiff has suffered a disfigurement and accordingly, the assertion that she meets the threshold established by s. 267.5(5) is limited to the provisions of sub-paragraph (b). Having regard to the definitions found in section 4.2(1) of Bill 198, and on the particular facts of this case⁵, to meet the threshold the Plaintiff must demonstrate:

- (i) *A serious impairment* – one that substantially interferes with her ability to continue her regular or usual employment, despite reasonable efforts to accommodate her and despite her reasonable efforts to use the accommodation to

² *Lento v. Castaldo* (sub nom. *Meyer v. Bright*) (1993), 15 O.R. (3d) 129 (C.A.).

³ See *Sabourin v. The Dominion of Canada General Insurance Company*, 2009 CarswellOnt. 1880 at para. 83.

⁴ *Peterson v. Phillips*, 2008 CarswellOnt. 2501 at paragraph 7.

⁵ Subsections 4.2(1)1ii and 2ii refer to circumstances where a plaintiff is unable to continue in a career training program, which is not the case here and so I have not summarized those provisions in para. 6.

allow her to continue employment⁶; or, one that substantially interferes with most of the usual activities of daily living, considering the Plaintiff's age⁷;

- (ii) *Of an important function* – one that is necessary to perform the activities that are essential tasks of her regular or usual employment, taking into account reasonable efforts to accommodate her and her reasonable efforts to use the accommodation⁸; or, one that is important to the usual activities of daily living, considering her age⁹; and,
- (iii) *That is permanent* – one that has been continuous since the accident and must, based on medical evidence and subject to the Plaintiff reasonably participating in the recommended treatment of the impairment, be expected not to substantially improve, must continue to meet the criteria of a serious impairment and must be expected to continue without substantial improvement when sustained by persons in similar circumstances.¹⁰

POSITIONS OF THE PARTIES:

[7] The Plaintiff alleges that she suffered permanent serious impairments of important physical and psychological functions as a result of injuries suffered in the collision on July 20, 2007. She asserts that she is unable to continue in any form of employment or to engage in her usual activities of daily living. She says she has been impaired in the same fashion from the date of the accident and expects to be impaired in the same fashion on a permanent basis. She asserts that her position is supported by her family doctor, as well as a consulting physiatrist, Dr. Ko, and a consulting psychiatrist, Dr. Rosenblat.

[8] The Defendant, on the other hand, denies that the Plaintiff has suffered any permanent serious impairment of any important physical, mental or psychological function. The Defendant asserts that there is no objective evidence of a physical problem with the Plaintiff and there is no medical restriction on her returning to work and/or her usual daily activities. With particular emphasis on the component elements of the threshold, the Defendant argues that:

- (i) There is conflicting evidence about what the Plaintiff can and/or can not do in terms of the usual activities of daily living. She has failed to establish, on a balance of probabilities, that her alleged injuries substantially interfere with those activities;
- (ii) The Plaintiff has made no effort to seek any type of employment at all and has not made any effort to seek out or attempt to use any form of accommodation;

⁶ s. 4.2(1) 1i.

⁷ s. 4.2(1) 1iii.

⁸ s. 4.2(1) 2i.

⁹ s. 4.2(1) 2iv.

¹⁰ s. 4.2(1) 3i-iii.

- (iii) The evidence supports the assertion that the Plaintiff suffers from intermittent pains only, consistent with her pre-accident history; and,
- (iv) There are inconsistencies in the Plaintiff's reporting of symptoms among the physicians she has consulted. For that reason, arguably among others, it is important to focus on the family doctor's notes and records, which reflect significant gaps in reporting of pain symptoms, consistent with the assertion that any injuries sustained by the Plaintiff in the accident have fully resolved.

THE EVIDENCE:

The Plaintiff

[9] The Plaintiff testified that, as a result of the accident, everything in her life has changed. She said that before the accident, her job was good, her health was good and her family life was good. Post-accident, everything has changed and now none of those things are good.

[10] She said that in the first weeks following the accident she could not move her neck at all. She could not sleep or eat. She was vomiting and having trouble going to the bathroom. Several weeks following the accident, she began experiencing and complaining of low back pain. She has also suffered headaches regularly since the accident. She had a history of migraine headaches prior to the accident which were mostly controlled by use of the drug Topomax. Since the accident the frequency and intensity of her migraine headaches has increased.

[11] She said she has continued to have significant neck and back pain since the accident. She has been unable to work. She is unable to do normal housekeeping functions and relies on her mother to assist her with vacuuming, changing linens, carrying heavy laundry and various other functions depending on her condition, which fluctuates from day-to-day. She is no longer able to engage in sexual activity with her husband.

[12] She said she has not been able to return to work since the accident. She has cashed in her RRSPs and now has significant debt, whereas she had no debt prior to the accident.

[13] Perhaps most troubling for the Plaintiff is her inability to participate in activities with her 11 year old daughter, Alex, as she was able to do prior to the accident. Alex now spends most weekends with the Plaintiff's mother and the Plaintiff relies on her mother and sister to engage in activities with Alex.

[14] The Plaintiff testified that she cries every day. She has disturbed sleep, problems with memory and difficulties with concentration.

The Plaintiff's Family Members

[15] The Plaintiff's common law husband, Tim Robertson, testified that following the accident, the Plaintiff has been unable to do many of the things she used to do, or at least do them the way she used to do them. At the time of the accident they were living as a family in Nobleton, Ontario but as a result of injuries sustained in the accident, the Plaintiff had difficulty

getting up and down the stairs to their loft. In the result, they moved to Brighton, Ontario to be nearer to the Plaintiff's mother, who was available to assist her.

[16] Mr. Robertson said the Plaintiff has been unable to participate in the activities she once did. She can go on walks in the park with him, but not for long. They can visit family together and they can go shopping together, but the Plaintiff is unable to stand for long periods at a time. She has difficulty with household tasks. For instance, she is unable to stand and do dishes without experiencing discomfort.

[17] He also testified that the Plaintiff generally complains about back pain. She mopes around a lot and cries daily. They no longer sleep together. The Plaintiff does not work and the reduction in household income has created a strain on their finances. He has taken on additional work assignments to try to make more money.

[18] The Plaintiff's younger sister testified and she described the Plaintiff, pre-accident, as a person who loved life, loved to work and loved to spend time with friends. She described the Plaintiff, post-accident, as being unable to do much, unless she is on pain killers, which she uses sparingly. While she could not think of anything specific that the Plaintiff can no longer do, she said that the Plaintiff is unable to do anything for a prolonged period.

[19] She went on to describe the Plaintiff as no longer independent. Instead she is reliant on others for a lot of help. Their mother provides a great deal of assistance. She herself travels to Brighton on weekends and she does things with Alex that the Plaintiff can no longer do. She said the Plaintiff is totally different now. She is a very depressed person who cries.

[20] The Plaintiff's mother also testified. She said that the Plaintiff was a very bright and hard working person who was part of the "in" crowd in high school. She was a devoted mother to her daughter and the two of them spent a lot of time together. She said the Plaintiff was in very good health prior to the accident and did not have a history of missing time from work.

[21] When she first saw the Plaintiff after the accident, she looked like an invalid. A short time later a decision was made that the Plaintiff's family would move, temporarily, to Brighton. The idea was to have the Plaintiff close by so that her mother could assist her as needed. That move became permanent when it appeared that the Plaintiff's injuries were not healing.

[22] The injuries have taken over the Plaintiff's life and have changed who she was. She lost her friends because she wouldn't commit to doing things with them on weekends. She and her husband used to be fun. Now it's more like business. She said that 99% of weekends Alex is with her because the Plaintiff isn't able to do the things with her that she used to do.

[23] She sees the Plaintiff virtually every day. There are some days she can do things and other days she can not. She said that before the accident the Plaintiff's life was 97% busy. Now her quality of life is not very good.

The Medical Evidence

[24] Dr. Weicker has been the Plaintiff's family physician for more than 10 years. She is the only treating physician to have testified at the trial. She said that while the Plaintiff did make complaints about low back pain and joint pain in the ten years prior to the trial, those complaints were intermittent. The Plaintiff had migraine headaches prior to the accident, but they were under control. She had also experienced incidents of low mood, but before the accident had not complained of low mood since April 2003.

[25] In the months leading up to the accident, the Plaintiff was, according to Dr. Weicker, in good health, physically and emotionally.

[26] Dr. Weicker saw the Plaintiff shortly following the accident, on July 23, 2007 at which time the Plaintiff complained of discomfort. She saw her again on July 30, 2007 and the Plaintiff at that time complained of pain between her shoulder blades and up into her neck. She noted, at that time, that the Plaintiff was unable to work, do housework and was not able to caregive.

[27] By August 9, 2007, Dr. Weicker noted that the Plaintiff was experiencing a lot of stress because she was unable to care for her daughter due to pain. She was attending physiotherapy three times per week. Her neck was 10% of normal and she had been having pain down to her low back for about one week.

[28] The Plaintiff's reports of pain continued into September 2007 despite her regular attendance at physiotherapy. Dr. Weicker's impression was that she was suffering from neck and lumbar strain. Her notes reflect that by January 16, 2008, the Plaintiff's neck was about 80% better and she said her back was functioning at about 50% of normal.

[29] She has continued to see the Plaintiff regularly since the accident. Her reports of pain have persisted, although they have varied, to the present. The Plaintiff complained of persistent back pain on September 1, 2009, November 18, 2009, March 3, 2010, July 20, 2010, October 28, 2010, November 15, 2010, December 27, 2010 and March 30, 2011. She regularly renewed a prescription for Tylenol 3 to help control the Plaintiff's pain.

[30] Her current impression is that the Plaintiff suffers from chronic back pain that began with the motor vehicle accident. It was Dr. Weicker's view that the Plaintiff will never be able to return to work.

[31] Dr. Ko was qualified, on consent, as an expert in physiatry. He conducted a three hour examination of the Plaintiff in which a number of physical tests were done. He also relied on a series of physical tests – largely range of motion tests - conducted by a chiropractor he works in conjunction with. He concedes that, for the most part, he had to rely on the Plaintiff's own self-reporting of pain and limitations. He attempts, however, to be as objective as possible in his examination. He did have available to him an MRI from January 2011 that showed some disc bulging. He acknowledged that disc bulging is a natural part of the aging process for many people, but his view was that the accident triggered the bulging in the Plaintiff's case.

[32] He found that the Plaintiff met the prevailing criteria for a diagnosis of fibromyalgia. He ultimately diagnosed her with the following conditions: diffuse chronic pain syndrome; probable cervical and lumbar facet syndrome; cervical, thoracic and lumbar strain and sprain;

cervicogenic/post-traumatic headaches; biomechanical joint dysfunction; and, post-traumatic myofascial pain syndrome.

[33] In his opinion, the Plaintiff has significant musculoskeletal impairment. He believes she has suffered injuries to her connective tissues. Her joints have been pushed beyond the normal physiological limit of the discs, tendons and/or ligaments to the point of causing microscopic tearing. If it were simple muscle strain, the muscles would heal. But he said what does not heal are ligaments and unhealed ligaments cause chronic pain. Moreover, he said that after a prolonged period of pain, a person's nervous system and brain begin to react differently and the ability to block pain responses becomes lessened.

[34] He said the Plaintiff's prognosis is guarded, which means it's not likely that she is going to recover on her own. He described her injury as a serious permanent impairment of an important physical function. She is not, in his opinion, likely to improve much – her impairment will not significantly change. She has reduced range of motion and pain affecting her sleep. She has lost the comfortable use of her upper back and neck area. She can't sit in a fixed position and is unable to engage in tasks that are both fine detailed hand motion and repetitive motions. He said it is unlikely she will ever be able to return to full-time employment. Her ability to return to part-time employment at some future point is less clear. In direct examination he testified that if she followed his recommendations she might be able to return to part-time employment. On cross-examination, however, he indicated that he agreed with the opinion of a kinesiologist who had previously opined that she was able to return to part-time employment now.

[35] It was Dr. Ko's evidence that the Plaintiff's injuries are all related to the motor vehicle accident on July 20, 2007.

[36] Dr. Rosenblat was qualified, on consent, as an expert in psychiatry.

[37] Dr. Rosenblat diagnosed the Plaintiff with major depressive episode, anxiety disorder and pain disorder, all of which he thought were caused by the accident. He thought there was some prospect that her depression would improve with treatment, but it was his view that her anxiety and pain disorders would not improve. In his opinion, the Plaintiff's condition is severe and she has a substantially reduced quality of life. She has the following impairments:

- (i) An inability to work;
- (ii) Some issues with personal hygiene;
- (iii) An inability to do day to day chores;
- (iv) Sadness, irritability, insomnia, loss of interest, fatigue;
- (v) Loss of sexual function; and,
- (vi) Decreased social functioning.

[38] In Dr. Rosenblat's opinion, she will never be able to return to work.

[39] Dr. Cameron was qualified, on consent, as an expert in orthopaedic surgery. His opinion was markedly different than those expressed by Dr. Weicker, Dr. Ko and Dr. Rosenblat. It was his view that the Plaintiff was exaggerating her symptoms. His opinion was that she had a soft

tissue sprain. She had no objective evidence of organic pathology and only subjective complaints. The Plaintiff's neurological examination was normal. In his view, there is no medical reason she can not resume any activity she wishes and she can resume normal household activities.

[40] Dr. Cameron admitted, under cross-examination, that up to 15% of people with soft tissue injuries do not recover within the time frame expected, or at all. He accepted that chronic pain syndrome does exist and that some people can have chronic pain with no readily observable objective basis for it. He conceded that he had not read the reports of Dr. Ko or Dr. Rosenblat and acknowledged that he made no comment on the Plaintiff's psychiatric condition.

DISCUSSION:

[41] As I have advised counsel orally, I have concluded that the Plaintiff did sustain a permanent, serious impairment of important physical and psychological functions.

[42] In reaching this conclusion, I have followed the stepped approach directed by the Court of Appeal in *Lento v. Castaldo*¹¹. Although *Lento* was decided a decade before Bill 198 came into effect, the stepped analysis remains appropriate, albeit informed now by the definitions provided by the regulation.¹² The approach to be taken involves answering the following questions in sequential fashion:

1. Has the injured person sustained permanent impairment of a bodily function caused by continuing injury which is physical, mental or psychological in nature?
2. If the answer to question 1 is yes, is the function, which is permanently impaired, an important one?
3. If the answer to question 2 is yes, is the permanent impairment of the important bodily function serious?

[43] I will review these three steps in turn. Before I do so, however, it is necessary to say something about the credibility and reliability of the Plaintiff's evidence.

[44] The medical opinion evidence in this case was heavily reliant on the Plaintiff's self-reporting of pain and limitations, so her credibility and reliability are squarely in issue. I pause to note that credibility and reliability are not the same thing. Credibility relates to the veracity of a witness. Reliability relates to the accuracy of the witness's testimony and engages a consideration of the witness's ability to accurately observe, recall and recount events in issue.¹³

¹¹ *Supra*, note 2.

¹² See *Sherman v. Guckelsberger*, 2008 CarswellOnt. 7969 (S.C.J.); *Ali v. Consalvo*, 2009 CarswellOnt. 604 (S.C.J.).

¹³ *R. v. C. (H.)*, 2009 ONCA 56, at para. 41.

[45] The Defendant urges the court to find that the Plaintiff was not a credible and reliable witness. He says that there is conflicting evidence about how much she can do and what she is doing. He relies heavily on the notes of Dr. Weicker, and in fact submitted to the court that those notes are the most reliable evidence in the record, given that the family doctor was the only treating physician to testify and given that the records cover a period of time from 2000 to the present. He says that there are inconsistencies between the reports made by the Plaintiff to the family doctor and reports she made to other experts, including Dr. Rosenblat and Dr. Ko.

[46] There are indeed some inconsistencies in the reports made by the Plaintiff to Dr. Rosenblat and Dr. Ko when compared to the family doctor's notes. For instance, the Plaintiff told Dr. Ko that she had not had any significant musculoskeletal pains in her pre-accident history. Dr. Weicker's notes, however, reflect that the Plaintiff complained of musculoskeletal pain as far back as 1999. There were further references in her notes to complaints of back pain in 2002 and again in October 2005. There was also a reference to knee pain on October 27, 2004 and another on August 25, 2005 when she noted arthritis symptoms in the left knee. Dr. Weicker's view was that the complaints pre-accident were intermittent. It may very well be, under the circumstances, that the Plaintiff was accurate when she said she had not had any *significant* musculoskeletal pains prior to the accident.

[47] In March 2010, the Plaintiff completed a neck disability index for her physiotherapist and noted that she could "concentrate fully with slight difficulty." She subsequently filled out a similar form for Dr. Ko where she noted that she had "great difficulty concentrating when she wanted to." Dr. Ko pointed out that the forms were filled out on different dates. He also said that it is more important to look at the total score on the neck disability index rather than on any one particular feature of the form. Her scores were 52 and 56 on the two different dates and so, in his view, they were fairly consistent.

[48] More significantly perhaps, Dr. Rosenblat testified that the Plaintiff told him she had not had any prior mental health issues. Dr. Weicker's notes make it clear, however, that she had been diagnosed with depression in October 2002 and had a further consultation about depression in April 2003. That said, there were no further notations of depression or any other mental health problems until after the accident.

[49] In my view, while there are inconsistencies between the Plaintiff's reports to Dr. Weicker and her reports to other consulting physicians, the inconsistencies are relatively minor and do not detract from the main points of her evidence. The Defendant urged me to rely on Dr. Weicker's notes to support the assertion that the Plaintiff's pains are intermittent. But Dr. Weicker's testimony was not consistent with that conclusion and moreover, Dr. Weicker's own conclusion was that the Plaintiff will never be able to return to work.

[50] Ultimately, I found the Plaintiff to be a credible witness. Her evidence was consistent as she told it, both internally and, for the most part, externally. It was supported by Dr. Ko and Dr. Rosenblat. Her family members corroborated her limitations. She was not shaken on cross-examination. Her evidence was also consistent with the probabilities of the case on the whole.

As O'Halloran J.A. famously said in *Faryna v. Chorny*¹⁴, the real test of an interested witness's credibility "must be its harmony with the preponderance of the probabilities which a practical and informed person would readily recognize as reasonable in that place and in those conditions."

[51] The Plaintiff has not worked since the date of the accident. Prior to then she had a consistent and uninterrupted work history dating back to her high school days. She has since had to cash in her RRSPs and is now in significant debt. Her husband has had to take on additional work to help make ends meet. She no longer engages as she once did with her young daughter, Alex, who spends almost every weekend with her grandmother now. She has lost friends. She has a well-documented depression.

[52] This is not a malingerer or faker. The Plaintiff's pain is real and her evidence, in my view, was genuine and compelling. I have no hesitation in relying upon it. I turn then to an analysis of the three steps directed in *Lento*.

Permanent Impairment

[53] Dr. Weicker testified that the Plaintiff has chronic pain. Dr. Ko agreed and said she has, amongst other things, chronic diffuse pain syndrome. His prognosis for the Plaintiff was guarded. She will not, in his opinion, likely improve much. Dr. Rosenblat testified that the Plaintiff suffers from three significant psychiatric impairments. While the Plaintiff may realize some improvement in terms of her depression, she will likely not improve in terms of her anxiety and pain disorder.

[54] Dr. Cameron did not believe that the Plaintiff suffered a permanent impairment. That said, I prefer the evidence of the Plaintiff's witnesses for the following reasons: (1) Dr. Cameron agreed that up to 15% of people with soft tissue injuries do not recover as expected. He accepted that chronic pain syndrome can exist in the absence of an objectively identifiable cause; (2) he completed only a brief examination of the Plaintiff – much less comprehensive than the examinations of Dr. Ko and Dr. Rosenblat; and, (3) he did not review the reports of Dr. Ko and Dr. Rosenblat and was unable to comment on them. Moreover, his report said nothing about any psychological impairment(s).

[55] I find, on a balance of probabilities, that the Plaintiff has suffered permanent impairments, both physical and psychological. She suffers from pain disorder and anxiety which will likely be permanent. Her depression may also be permanent. She also has chronic back and neck pain.

[56] Her impairments have been continuous since the accident. Some days are better than others as she admitted. Sometimes the pain is worse in her neck. Other times it is worse in her back. Dr. Ko testified that chronic pain tends to fluctuate from day to day. A patient may feel better in one part one day but have loading in another part. It's a very difficult condition to treat

¹⁴ [1951], B.C.J. No. 129 (B.C.C.A.)

for that reason. In my view, the variable or fluctuating nature of the Plaintiff's pain does not mean that it is not a continuous impairment.

[57] The Plaintiff has diligently pursued medical attention and has engaged in significant courses of physiotherapy, without resolution. Both Dr. Ko and Dr. Rosenblat opined that, notwithstanding medical treatment, the Plaintiff's impairments will continue.

Of an Important Function

[58] Important functions are those that are necessary to perform the essential tasks of regular or usual employment, taking into account reasonable efforts to accommodate, or ones important to the usual activities of daily living.

[59] The Plaintiff's psychological impairments affect her memory and her concentration. They cause sleep disturbances, as does her chronic pain, which in turn also affect concentration. She is unable to multi-task and has little productive activity in a day. Her condition, according to Dr. Rosenblat's uncontradicted evidence, is severe. She is unable to work full or part-time in any occupation. I am satisfied, on Dr. Rosenblat's evidence alone, that the Plaintiff's psychological impairments affect functions necessary to perform the essential tasks of regular employment and also to perform the usual activities of daily living.

[60] Beyond Dr. Rosenblat's evidence, the Plaintiff's inability to sit or stand for prolonged periods, the loss of the comfortable use of her upper back and neck area and the inability to engage in tasks that are both fine detailed hand motions and repetitive motions also affect the functions necessary to perform the essential tasks of regular employment. She can sometimes perform many of the usual activities of daily living, but quite frequently she can not. She requires the ongoing assistance of her mother to perform many of the tasks of housekeeping. She is limited in what she can do in terms of extra-curricular activities as well.

That is Serious

[61] A serious impairment is one that substantially interferes with the Plaintiff's ability to continue her regular or usual employment, despite reasonable efforts to accommodate her, or one that substantially interferes with most of the usual activities of daily living, considering the Plaintiff's age.

[62] For reasons I have already set out, I am satisfied that the Plaintiff's impairments – both psychological and physical, *substantially*, if not entirely, interfere with her ability to continue her usual employment. Defence counsel argued that she had not pursued any form of accommodation. While that is true, I am satisfied, on the basis of the evidentiary record, that the Plaintiff's impairments can not reasonably be accommodated. Even if her physical limitations could be accommodated – and major accommodations would be necessary – her psychological impairments can not. Again, Dr. Rosenblat testified that, in his opinion, the Plaintiff will not be able to return to work.

[63] Moreover, on the basis of the evidence of the Plaintiff, her husband, sister and mother, which I accept, her physical impairments substantially interfere with her usual activities of daily living.

CONCLUSION:

[64] I am, in summary, satisfied that the injuries suffered by the Plaintiff in the collision on July 20, 2007, resulted in both physical and psychological impairments of important functions that are serious and permanent within the meaning of those terms as defined in Bill 198.

[65] In the result, the Plaintiff is permitted to pursue damages for non-pecuniary losses under section 267.5(5) of the *Insurance Act*.

Boswell J.

Date: August 3, 2011

CORRIGENDA

1. K. Lasko has been added to counsel's name, for the Defendant.

Appendix “A”

DEFINITION OF PERMANENT SERIOUS IMPAIRMENT OF AN IMPORTANT PHYSICAL, MENTAL OR PSYCHOLOGICAL FUNCTION

4.1 For the purposes of section 267.5 of the Act,

“permanent serious impairment of an important physical, mental or psychological function” means impairment of a person that meets the criteria set out in section 4.2. O. Reg. 381/03, s. 1.

4.2 (1) A person suffers from permanent serious impairment of an important physical, mental or psychological function if all of the following criteria are met:

1. The impairment must,
 - i. substantially interfere with the person’s ability to continue his or her regular or usual employment, despite reasonable efforts to accommodate the person’s impairment and the person’s reasonable efforts to use the accommodation to allow the person to continue employment,
 - ii. substantially interfere with the person’s ability to continue training for a career in a field in which the person was being trained before the incident, despite reasonable efforts to accommodate the person’s impairment and the person’s reasonable efforts to use the accommodation to allow the person to continue his or her career training, or
 - iii. substantially interfere with most of the usual activities of daily living, considering the person’s age.
2. For the function that is impaired to be an important function of the impaired person, the function must,
 - i. be necessary to perform the activities that are essential tasks of the person’s regular or usual employment, taking into account reasonable efforts to accommodate the person’s impairment and the person’s reasonable efforts to use the accommodation to allow the person to continue employment,
 - ii. be necessary to perform the activities that are essential tasks of the person’s training for a career in a field in which the person was being trained before the incident, taking into account reasonable efforts to accommodate the person’s impairment and the person’s reasonable efforts to use the accommodation to allow the person to continue his or her career training,
 - iii. be necessary for the person to provide for his or her own care or well-being, or
 - iv. be important to the usual activities of daily living, considering the person’s age.

3. For the impairment to be permanent, the impairment must,
 - i. have been continuous since the incident and must, based on medical evidence and subject to the person reasonably participating in the recommended treatment of the impairment, be expected not to substantially improve,
 - ii. continue to meet the criteria in paragraph 1, and
 - iii. be of a nature that is expected to continue without substantial improvement when sustained by persons in similar circumstances. O. Reg. 381/03, s. 1.

(2) This section applies with respect to any incident that occurs on or after October 1, 2003. O. Reg. 381/03, s. 1.

EVIDENCE ADDUCED TO PROVE PERMANENT SERIOUS IMPAIRMENT OF AN IMPORTANT PHYSICAL, MENTAL OR PSYCHOLOGICAL FUNCTION

4.3 (1) A person shall, in addition to any other evidence, adduce the evidence set out in this section to support the person's claim that he or she has sustained permanent serious impairment of an important physical, mental or psychological function for the purposes of section 267.5 of the Act. O. Reg. 381/03, s. 1.

(2) The person shall adduce evidence of one or more physicians, in accordance with this section, that explains,

- (a) the nature of the impairment;
- (b) the permanence of the impairment;
- (c) the specific function that is impaired; and
- (d) the importance of the specific function to the person. O. Reg. 381/03, s. 1.

(3) The evidence of the physician,

- (a) shall be adduced by a physician who is trained for and experienced in the assessment or treatment of the type of impairment that is alleged; and
- (b) shall be based on medical evidence, in accordance with generally accepted guidelines or standards of the practice of medicine. O. Reg. 381/03, s. 1.

(4) The evidence of the physician shall include a conclusion that the impairment is directly or indirectly sustained as the result of the use or operation of an automobile. O. Reg. 381/03, s. 1.

(5) In addition to the evidence of the physician, the person shall adduce evidence that corroborates the change in the function that is alleged to be a

permanent serious impairment of an important physical, mental or psychological function. O. Reg. 381/03, s. 1.

(6) This section applies with respect to any incident that occurs on or after October 1, 2003. O. Reg. 381/03, s. 1.